Insurance

Company Name:
Plan Name:
Address:

Phone Number:
Fax Number:
Contact Person:
Plan Type:
Group Name:
Policy Number:
Group Number:
Start/End Date:
Copay:

Home Phone:
Work Phone:
Mobile Phone:
Other Phone:
Personal Email:
Work Email:
Other Email:

*Recent Appointments**

Date/Time of Visit	*Provider/Resource*	*Visit Type*	*Reason for Visit*	*Status*
04/14/2021 07:30 AM	Justin Merryman	Patient	Continued Care	CheckedOut
03/31/2021 09:00 AM	Justin Merryman	Patient	Continued Care	CheckedOut
03/17/2021 12:45 PM	Justin Merryman	Patient	Continued Care	CheckedOut
03/15/2021 09:15 AM	Justin Merryman	Patient	Continued Care	NoShow
03/01/2021 12:00 PM	Justin Merryman	Patient	Continued Care	CheckedOut
04/28/2021 07:30 AM	Justin Merryman	Patient	Continued Care	CheckedOut
12/28/2020 09:45 AM	Justin Merryman	Patient	Continued Care	CheckedOut
12/14/2020 01:00 PM	Justin Merryman	Patient	Continued Care	CheckedOut
12/09/2020 10:45 AM	Justin Merryman	Patient	Continued Care	CheckedOut
12/02/2020 09:15 AM	Justin Merryman	Patient	Continued Care	Cancelled
02/18/2021 06:00 PM	Justin Merryman	Patient	Continued Care	CheckedOut
02/12/2021 07:30 AM	Justin Merryman	Patient	Continued Care	CheckedOut
02/11/2021 06:00 PM	Justin Merryman	Patient	Continued Care	Rescheduled
02/03/2021 11:00 AM	Justin Merryman	Patient	Continued Care	CheckedOut
01/27/2021 10:45 AM	Justin Merryman	Patient	Continued Care	CheckedOut
01/20/2021 11:15 AM	Justin Merryman	Patient	Continued Care	CheckedOut
01/15/2021 08:30 AM	Justin Merryman	Patient	Continued Care	CheckedOut
01/13/2021 09:45 AM	Justin Merryman	Patient	Continued Care	CheckedOut
11/13/2020 09:30 AM	Justin Merryman	Patient	Report of Findings	Rescheduled
11/11/2020 12:00 PM	Justin Merryman	Patient	Report of Findings	CheckedOut
11/06/2020 09:00 AM	Justin Merryman	Patient	Continued Care	CheckedOut
10/30/2020 09:00 AM	Justin Merryman	Patient	Report of Findings	CheckedOut
10/23/2020 08:00 AM	Justin Merryman	Patient	New Patient Consultation	CheckedOut

*NOTE: Please keep in mind that if we were not to able retrieve appointments, we will display an empty list. Max 50 appointments total that includes the next 10 appointments within a year plus the most recent in the last 2 years.

FACE SHEET

Viktoriya A Pulliam
MRN: NA
DOB: 06 Dec 1989 (31 yo)
Female

Allergies

- No allergy history has been documented for this patient.

Medications

- No medications documented for this patient.

Problems

- No active Problems listed for this patient

History

PMHx

- No past medical history has been documented for this patient

PSHx

- No past surgical history has been documented for this patient

FHx

- No family history has been documented for this patient

SHx

- No social history has been documented for this patient

OB & Preg Hx

- No Ob or Preg history has been documented for this patient

Hospitalizations

- No hospitalizations / procedures have been documented for this patient

ImplantableDevices

- No implantable devices has been documented for this patient

PATIENT CHART

Problems List

Active Problems

- No active problems listed for this patient.

Inactive Problems

- No inactive problems listed for this patient.

Allergies

Active Allergies

- No allergy history has been documented for this patient.

Inactive Allergies

- No allergy history has been documented for this patient.

Medications List

Active Medications

- No medications documented for this patient.

Discontinued Medications

- No medications documented for this patient.

Vital Signs

RECORDED	BP	HR	RR	TEMP	HT/LT.	WEIGHT	BMI	HEAD CIRC	SpO2	INHALED O2	COMMENT

Labs

- No lab documentations found for this patient.

Notes

DATE	TYPE	SUBJECT	STATUS	ASSIGNED TO	Signed By	LAST UPDATED
28 Apr 2021 07:30 AM	SOAP Note 1		Complete	Justin Merryman	Justin Merryman	28 Apr 2021 05:39 PM
14 Apr 2021 07:46 AM	SOAP Note 1		Complete	Justin Merryman	Justin Merryman	14 Apr 2021 07:59 AM
31 Mar 2021 09:21 AM	SOAP Note 1		Complete	Justin Merryman	Justin Merryman	31 Mar 2021 09:22 AM
17 Mar 2021 12:45 PM	SOAP Note 1		Complete	Justin Merryman	Justin Merryman	17 Mar 2021 03:16 PM
01 Mar 2021 12:00 PM	SOAP Note 1		Complete	Justin Merryman	Justin Merryman	01 Mar 2021 12:28 PM
18 Feb 2021 06:00 PM	SOAP Note 1		Complete	Justin Merryman	Justin Merryman	18 Feb 2021 06:54 PM
12 Feb 2021 07:30 AM	SOAP Note 1		Complete	Justin Merryman	Justin Merryman	12 Feb 2021 12:04 PM
03 Feb 2021 11:00 AM	SOAP Note 1		Complete	Justin Merryman	Justin Merryman	03 Feb 2021 12:23 PM
27 Jan 2021 10:45 AM	SOAP Note 1		Complete	Justin Merryman	Justin Merryman	27 Jan 2021 11:44 AM
20 Jan 2021 11:15 AM	SOAP Note 1		Complete	Justin Merryman	Justin Merryman	20 Jan 2021 11:41 AM
15 Jan 2021 08:30 AM	SOAP Note 1		Complete	Justin Merryman	Justin Merryman	15 Jan 2021 08:41 AM
13 Jan 2021 09:45 AM	SOAP Note 1		Complete	Justin Merryman	Justin Merryman	13 Jan 2021 10:13 AM
28 Dec 2020 09:45 AM	SOAP Note 1		Complete	Justin Merryman	Justin Merryman	28 Dec 2020 10:02 AM
14 Dec 2020 01:00 PM	SOAP Note 1		Complete	Justin Merryman	Justin Merryman	14 Dec 2020 01:11 PM
09 Dec 2020 10:45 AM	SOAP Note 1		Complete	Justin Merryman	Justin Merryman	09 Dec 2020 10:46 AM
11 Nov 2020 12:00 PM	SOAP Note 1		Complete	Justin Merryman	Justin Merryman	11 Nov 2020 12:19 PM
06 Nov 2020 09:00 AM	SOAP Note 1		Complete	Justin Merryman	Justin Merryman	06 Nov 2020 09:13 AM
30 Oct 2020 09:00 AM	SOAP Note 1		Complete	Justin Merryman	Justin Merryman	30 Oct 2020 11:02 AM
23 Oct 2020 08:00 AM	SOAP Note 1		Complete	Justin Merryman	Justin Merryman	23 Oct 2020 10:47 AM

SOAP Note 1 Note

Viktoriya A Pulliam
DOB: 06 Dec 1989 (31 yo Female)

Visit Date: 23 Oct 2020

Subjective

Severe as Viktoriya has been involved in in multiple car accidents. After accident she has noticed severe spinal arthralgia affecting both the cervical thoracic and lumbar spine. She is working with a chiropractor back home in Oregon and there again to a point where her symptoms were easier to deal with. During her transition to Wyoming she has started a position where she has been on her feet for long periods of time and she has been having bilateral radicular findings in the low back along with increased cervicalgia. A lot of this she contributes to the car accidents as prior to the accidents when she would be standing for periods of time or working with computer she did not have the same symptoms. Both car accidents were a high-velocity impact there was no bracing affected done lots of passive impact to the spine

Objective

Today's objective findings: Positive Spurling's on the left C1 and C5 causing mild pulling into the levator scapula. Decompression within normal limits SLR within normal limits positive Kemps bilateral shooting pain into the bilateral glutes.
Neurological testing bilateral extremities both strength and vibration and light sensation are all within normal limits
Prone spine testing direct facet compression testing increased pain at the cervicothoracic junction along with the lumbosacral junction approximately 7 out of 10 no radicular shooting.
Motion palpation detected segmental dysfunctions in the areas of the cervical thoracic and lumbosacral spine specifically left C5 left C1 T1 posterior T6 posterior right L5 right S1 and right iliac.
Severe hypertonicity bilateral paraspinals throughout the cervical thoracic and lumbar spine not very specific to the area. Trigger points detected bilateral levator scapula severe bilateral lumbar multifidus severe bilateral cervical multifidus severe.
Moderate decrease in range of motion in both hips and shoulder joints shoulder joints 25% reduction in internal rotation with pulling into the scapula creating 5 out of 10 on pain restriction into bilateral hips again with flexion and internal rotation about a 25% reduction causing about a 5 x 10 in pain
Lumbosacral radiographs were taken today a right transitional segment at the L5-S1 joint was found as far as the positioning of her spine on AP view its relatively straight with a little bit of leftward lean the lateral projection finds a moderate decrease in the L5-S1 disc space with a decrease in the L5-S1 foramina.

Assessment

The prognosis for Viktoriya is to be determined at this point in time. Due to the radicular findings in the low back I am sending out for an MRI to make sure that there is no foraminal encroachment. I am also requesting the cervical MRI.

Plan

Follow-up with the required imaging
Treatment today consisted of chiropractic manipulative treatment of the objective segments using a diversified technique
Trigger point dry needling of all objective tissue 5 minutes
Motion restriction stretching of all objection motion restriction 10 minutes
Follow-up in 1 week

Electronically signed by:
Justin Merryman, DC
Signed on: 23 Oct 2020 09:57 AM

Addendum

23 Oct 2020, 11:47 AM - Written by Justin Merryman, DC

Vitals were taken today but were left out of the note accidentally. Victoria is 5 foot 5 weighing approximately 150 pounds her systolic blood pressure is 121 her diastolic blood pressure is 78 and her pulse rate is 75

SOAP Note 1 Note

Viktoriya A Pulliam
DOB: 06 Dec 1989 (31 yo Female)

Visit Date: 30 Oct 2020

Subjective

Victoria is amazed by how batch better she feels after the dry needling her low back has loosened up quite a bit as long with her neck and shoulders.

Objective

Segmental dysfunctions as noted right L5-S1 and iliac T6 posterior right T1 right C7 left C5 left C1
Bilateral hypertonicity of the multifidus cervical and lumbar moderate to severe
Mild restriction both hips and shoulders internal rotation reduced by about 25%

Assessment

The prognosis for Viktoriya is to be determined at this point in time. Due to the radicular findings in the low back I am sending out for an MRI to make sure that there is no foraminal encroachment. I am also requesting the cervical MRI.

Plan

Follow-up with the required imaging
Treatment today consisted of chiropractic manipulative treatment of the objective segments using a diversified technique
Trigger point dry needling of all objective tissue 5 minutes
Motion restriction stretching of all objection motion restriction 10 minutes
Follow-up in 1 week

Electronically signed by:
Justin Merryman, DC
Signed on: 30 Oct 2020 11:02 AM

SOAP Note 1 Note

Viktoriya A Pulliam
DOB: 06 Dec 1989 (31 yo Female)

Visit Date: 06 Nov 2020

Subjective

Victoria is amazed by how batch better she feels after the dry needling her low back has loosened up quite a bit as long with her neck and shoulders.

Objective

Segmental dysfunctions as noted right L5-S1 and iliac T6 posterior right T1 right C7 left C5 left C1
Bilateral hypertonicity of the multifidus cervical and lumbar moderate to severe
Mild restriction both hips and shoulders internal rotation reduced by about 25%

Assessment

The prognosis for Viktoriya is to be determined at this point in time. Due to the radicular findings in the low back I am sending out for an MRI to make sure that there is no foraminal encroachment. I am also requesting the cervical MRI.

Plan

Follow-up with the required imaging
Treatment today consisted of chiropractic manipulative treatment of the objective segments using a diversified technique
Trigger point dry needling of all objective tissue 5 minutes
Motion restriction stretching of all objection motion restriction 10 minutes
Follow-up in 1 week

Electronically signed by:
Justin Merryman, DC
Signed on: 06 Nov 2020 09:13 AM

SOAP Note 1 Note

Viktoriya A Pulliam
DOB: 06 Dec 1989 (31 yo Female)

Visit Date: 11 Nov 2020

Subjective

Telemedicine call was placed to Victoria today to discuss her progressing symptoms and the MRI results lately she has been feeling better but that may be due to the fact she is not moving around a lot due to her quarantine.

Objective

Objective measurements at this point time are difficult due to telemedicine.

Assessment

Prognosis for Victoria has worsened slightly based on the positive MRI findings. We have discussed multiple different options for her both in terms of education and prognosis.

Plan

Once out of quarantine we will get Victoria back on schedule for conservative based treatment we also might try some route of physical therapy with McKenzie decompression.

Electronically signed by:
Justin Merryman, DC
Signed on: 11 Nov 2020 12:19 PM

SOAP Note 1 Note

Viktoriya A Pulliam
DOB: 06 Dec 1989 (31 yo Female)

Visit Date: 09 Dec 2020

Subjective

Victoria is amazed by how batch better she feels after the dry needling her low back has loosened up quite a bit as long with her neck and shoulders.

Objective

Segmental dysfunctions as noted right L5-S1 and iliac T6 posterior right T1 right C7 left C5 left C1
Bilateral hypertonicity of the multifidus cervical and lumbar moderate to severe
Mild restriction both hips and shoulders internal rotation reduced by about 25%

Assessment

The prognosis for Viktoriya is to be determined at this point in time. Due to the radicular findings in the low back I am sending out for an MRI to make sure that there is no foraminal encroachment. I am also requesting the cervical MRI.

Plan

Follow-up with the required imaging
Treatment today consisted of chiropractic manipulative treatment of the objective segments using a diversified technique
Trigger point dry needling of all objective tissue 5 minutes
Motion restriction stretching of all objection motion restriction 10 minutes
Follow-up in 1 week

Electronically signed by:
Justin Merryman, DC
Signed on: 09 Dec 2020 10:46 AM

SOAP Note 1 Note

Viktoriya A Pulliam
DOB: 06 Dec 1989 (31 yo Female)

Visit Date: 14 Dec 2020

Subjective

Victoria continues to improve with cervical arthralgia. Her lumbosacral arthralgia is having less radicular signs and symptoms but every once in a while if she bends quickly or twists just the wrong she will feel a shooting pain does not last long and it fades quickly.

Objective

Segmental dysfunctions as noted right L5-S1 and iliac T6 posterior right T1 right C7 left C5 left C1
Bilateral hypertonicity of the multifidus cervical and lumbar moderate to severe
Mild restriction both hips and shoulders internal rotation reduced by about 25%

Assessment

We are approaching a full reexamination on Victoria, she has made good progress and we have enough treatments under her belt to objectively gauge improvement. A lot of her improvement I believe is due to the fact that she is now more educated on her lumbosacral spine with the recent MRI that we did.

Plan

Follow-up with the required imaging
Treatment today consisted of chiropractic manipulative treatment of the objective segments using a diversified technique
Trigger point dry needling of all objective tissue 5 minutes
Motion restriction stretching of all objection motion restriction 10 minutes
Follow-up in 1 week

Electronically signed by:
Justin Merryman, DC
Signed on: 14 Dec 2020 01:11 PM

SOAP Note 1 Note

Viktoriya A Pulliam
DOB: 06 Dec 1989 (31 yo Female)

Visit Date: 28 Dec 2020

Subjective

Victoria went on vacation so activities of daily living are hard to gauge along with workplace ergonomics. As suspected during vacation she did not have much low back pain. One positive that she did mention is her radiation has not come back this is good news.

Objective

Segmental dysfunctions as noted right L5-S1 and iliac T6 posterior right T1 right C7 left C5 left C1
Bilateral hypertonicity of the multifidus cervical and lumbar moderate to severe
Mild restriction both hips and shoulders internal rotation reduced by about 25%

Assessment

I believe at this point we can say we have reached centralization of the pain. We are going to start extending time in between treatments to make sure she maintains her level of centralization and at that point therapeutic exercise will start to be implemented

Plan

Follow-up with the required imaging
Treatment today consisted of chiropractic manipulative treatment of the objective segments using a diversified technique
Trigger point dry needling of all objective tissue 5 minutes
Motion restriction stretching of all objection motion restriction 10 minutes
Follow-up in 1 week

Electronically signed by:
Justin Merryman, DC
Signed on: 28 Dec 2020 10:02 AM

SOAP Note 1 Note

Viktoriya A Pulliam
DOB: 06 Dec 1989 (31 yo Female)

Visit Date: 13 Jan 2021

Clinical Note

Victoria presented today with an acute exacerbation of a chronic whiplash related condition. Specifically in the right lower cervical spine she had an acute pinch during a weightlifting exercise that turned into a severe muscle spasm radiating to the medial border of her scapula is difficult for her head to turn from side to side and lift her right arm there is quite a bit of pain she recently got a massage which helped control the pain but the range of motion is still heavily restricted. Today's treatment consisted of chiropractic manipulative treatment of the objective segments the objective segments were as follows: Right C5 left C1 T1 posterior T6 posterior L5-S1 and iliac right. We also did 4 minutes of trigger point dry needling to the objective tissue moderate to severe tissue hypertonicity right levator Scap and right upper trap. We had good and cavitation during manipulation increase range of motion and decrease pain slightly good twitch response during dry needling reduce pain as well. At this point time the prognosis is good she is going to follow-up sooner approximately this Friday at that point time expect that we will get more range of motion as soon as we can get her back to a more baseline pain management area I am going to work with her on weight lifting exercises that do not encourage cervicothoracic hyperextension or pressure so that she does not impinge of those joints further.

Electronically signed by:
Justin Merryman, DC
Signed on: 13 Jan 2021 10:13 AM

SOAP Note 1 Note

Viktoriya A Pulliam
DOB: 06 Dec 1989 (31 yo Female)

Visit Date: 15 Jan 2021

Clinical Note

The pain was too severe after last treatment so she contacted her medical provider who prescribed her an oral corticosteroid. She is noticing a mild relief in pain a lot this could be due to the treatment and the oral corticosteroid combined. Overall today she feels like she has a little bit more range of motion but her low back is starting to become a little bit more achy especially when she is trying to sleep she is however heating the low back instead of icing before bed I told her that icing before bed is good to help move the inflammation around and away from the nerve were more concerned with the nerves in this case than the muscles because this is not a muscular strain type injury this is a somatic referral due to joint impingement on the medial branch nerves. Overall objectively C5 right C1 on the left T1 posterior T6 posterior right L5 right S1 right iliac all fixated. Muscle tonicity moderate to severe right lumbar multifidus right levator Scap. Treatment today consisted of chiropractic manipulative treatment of the objective segments using a diversified technique good audible pops even more than last time increased range of motion and more pain release on this treatment. Trigger point dry needling performed great with good twitch results we try to stay very minimal so that we did not create too much localized inflammation. Overall the treatment today went very well more successful than the last. Prognosis at this point time is stable and she should start feeling back to normal within the next few treatments. In the future we are going to walk her through some good exercises so that she does not create this joint impingement syndrome to keep coming back. Post whiplash related patients are difficult to manage in terms of bringing him back to exercise because most of the exercises they do are traditional and are therefore terrible for the joints not only the shoulder in the extremities but also due to the placement of the weight there is a lot of focal pressure in positions on the spine.

Electronically signed by:
Justin Merryman, DC
Signed on: 15 Jan 2021 08:41 AM

SOAP Note 1 Note

Viktoriya A Pulliam
DOB: 06 Dec 1989 (31 yo Female)

Visit Date: 20 Jan 2021

Clinical Note

Victoria presented with a mild improvement from last appointment. During workplace ergonomics she still feels that her left side gets inflamed on her neck and will eventually transition pain down her arm the backside of it this is both a somatic and joint related issue. Other than that her low back has been doing better there is no radicular symptoms whatsoever which is a huge improvement she has has centralized low back pain that might be a little bit more towards the right. Objectively today left T1 and C7 fixated mild to moderate trigger point levator Scap left. Mild to moderate trigger point right lumbar multifidus. Segmental dysfunction right L5 right S1 right iliac and T6 posterior. Postural positioning form today she has mild anterior forward head posture mostly she has a flattened thoracic spine but this only due to the fact that her scapula are weaning out more and its making it look that she has a flattened thoracic spine. Today we taught her corrective exercises for scapular protraction that she can do to help with that head fall into place. Along with that we did CMT of the objective segments getting a good release along with everything else we did trigger point dry needling of 4 minutes to the objective tissue with a good subjective improvement overall today she left saying that she feels better. Prognosis is great she is improving there is still a few things that we need to work on in terms of workplace ergonomics and strengthening and that is what organ be doing into the future.

Electronically signed by:
Justin Merryman, DC
Signed on: 20 Jan 2021 11:41 AM

SOAP Note 1 Note

Viktoriya A Pulliam
DOB: 06 Dec 1989 (31 yo Female)

Visit Date: 27 Jan 2021

Clinical Note

Left-sided radiculopathy improving slightly but with neck extension still aggravates it. Objective test today Spurling's is still positive on the left side there is also some tethering into the shoulder we worked to hypertonicity moderate to severe left infraspinatus along with left levator scapula. Segmental dysfunctions right see 1 left C5 left C6 left C7 left T1. CMT of the objective segments did reduce subjective level pain and stiffness. Overall intensity has decreased over time we are now focused more on the radiculopathy that she is getting during extension we are going to focus a little bit more on postural recoordination. Follow-up next week

Electronically signed by:
Justin Merryman, DC
Signed on: 27 Jan 2021 11:44 AM

SOAP Note 1 Note

Viktoriya A Pulliam
DOB: 06 Dec 1989 (31 yo Female)

Visit Date: 03 Feb 2021

Clinical Note

Victoria felt pretty defeated today [illegible] ks so were not truly sure if there is some conflicting in[illegible]ion going on here. In terms of her pain she is just irritated with the longevity that has been going on very common with chronic pain based offers. I told her today if iodine for every post whiplash related incident that it end up in chronic pain I would not work anymore. We are on track doing something good she understands that but we also have a consultation set up with the regenerative specialist I think this is a benefit to her to see if there is something more that could be done in a curative manner. Other than that I told her let start planning for treatments after she goes the clinic because clinic seems to be the part of the week where she gets the worst amount of pain and has a lot to do with the workplace ergonomics being very on adaptable to good posture. Today's treatment was the same as before without trigger point dry needling we did focus on some good postural-based exercises like scapular retraction head retraction forward posture crisscross 8 minutes. Chiropractic manipulative treatment was initiated into the body C5 right C1 left T6 posterior right L5 right S1 right iliac.

Electronically signed by:
Justin Merryman, DC
Signed on: 03 Feb 2021 12:23 PM

SOAP Note 1 Note

Viktoriya A Pulliam
DOB: 06 Dec 1989 (31 yo Female)

Visit Date: 12 Feb 2021

Clinical Note

Victoria presented feeling better today. Recent change in her medical history she has been having severe cramping secondary to menstrual related issues she has had a type topic pregnancy in her past so she is on top of that 1 she is getting her levels measured and I told her that an ultrasound may be necessary. Other than that her neck and low back pain have actually gotten better she decided to decline Dr. McGinley's consultation due to the fact that it costs $1000 I plan to refer her to Dr. Manzo who is a another specialist and pain medicine who offers regenerative therapy as well we will see how that goes she is obviously a little disgruntled with her treatment as this been going on for some time the unfortunate nature behind this as if I would have had her from the very get go I believe we would have not only not stronger along but when I would have been able to get her more of a well-developed prognosis sooner this stage she has been through so much that depression is definitely something Robbie's again to be dealing with in combination with the pain. Objectively today muscle tonicity full spinal everything is pretty tight fascial related restrictions we did a minor SF MA she is pretty much restricted in every plane motion with not serious pain we decided to do flexion distraction technique full myofascial release from head to toe and we are able to get audible pops during she says that that table seems to give her the greatest relief for the longest period time so we will continue with that.

Electronically signed by:
Justin Merryman, DC
Signed on: 12 Feb 2021 12:04 PM

SOAP Note 1 Note

Viktoriya A Pulliam
DOB: 06 Dec 1989 (31 yo Female)

Visit Date: 18 Feb 2021

Clinical Note

Today is a good day for Victoria she is feeling less pain overall she contributes a lot of this to doing less intense exercise but she also contributes it to other things that she does not know if it causes it or not which is related to hormonal type pathologies that she is going on with the right now with her menstrual cycle. She does have a consultation to be set up with Dr. Mandel a local pain management specialist I am using him as a out-of-the-box perspective to see if there is something more we could do to decrease the pain for a longer period of time at a conservative level without doing obviously any narcotic approach or surgical approach. In terms of progression she is also felt like the flexion distraction myofascial release has helped more than anything I can have to hit myself on the head because she has been starting with this from square one because there are a lot of fascial restrictions that she is dealing with mostly secondary to her job but all stemming from this car wreck and now that were working the full fascial planes she is feeling that she actually gets more free range of not only motion but just flexibility in general sleep has been doing better which contributes to her overall decrease in pain as well. Outside of the flexion distraction today for about 8 minutes we did trigger point dry needling bilateral upper traps for 4 minutes we also did scapular retraction exercise for 8 minutes in the adjusted right C5 left C1 T6 posterior right L5 right S1 right iliac all the treatment subjectively decreased her overall stiff and achy appearance although today was good I hope he continues to go in that manner we will check back up in 1 week.

Electronically signed by:
Justin Merryman, DC
Signed on: 18 Feb 2021 06:54 PM

SOAP Note 1 Note

Viktoriya A Pulliam
DOB: 06 Dec 1989 (31 yo Female)

Visit Date: 01 Mar 2021

Clinical Note

Victoria presents today in less pain than she has had on previous treatments she feels that the flexion distraction myofascial release has helped the most as far as activities of daily living her job is still pretty stressful and causes pain but she is not completely out of it when she goes home this is a great step forward she has also contacted the anesthesiologist that I wanted to and she is going to set up a consultation to see if there is something else that we can be doing to help with pain management and almost all of my pain management cases we need to cotreat and this is the next best option pain level wise fluctuates but she is noticing more and more with each passing day that it is more manageable she does not have any new injury or any new changes to her medical history. Objectively today active range of motion is pretty stable she has full range of motion of the lumbosacral and cervical spine with mild irritation passive range of motion is the same palpation of the tissues reveals mild tissue hypertonicity bilateral paraspinals lumbar to cervical spine levator Scap upper trap and piriformis Motion palpation of the specific segments right C5 left C1 and left SI are fixated assessment she is doing very well with the conservative care in fact we have reached what I would call a milestone in terms of pain management this is a perfect time for a coconsultation with a pain management specialist prognosis at this point time is better than it has been. Today's treatment consisted of myofascial release in the flexion distraction table for 8 minutes posterior head traction with Scap stabilization exercise physeal ball 8 minutes trigger point dry needling for 5 minutes see objective tissue and manipulation using diversified to the objective segments moving forward I believe that with a consultation with a pain specialist we will have more information and be able to manage her pain with a higher proficiency rate.

Electronically signed by:
Justin Merryman, DC
Signed on: 01 Mar 2021 12:28 PM

SOAP Note 1 Note

Viktoriya A Pulliam
DOB: 06 Dec 1989 (31 yo Female)

Visit Date: 17 Mar 2021

Subjective

Victoria presents today in less pain than she has had on previous treatments she feels that the flexion distraction myofascial release has helped the most as far as activities of daily living her job is still pretty stressful and causes pain but she is not completely out of it when she goes home this is a great step forward she has also contacted the anesthesiologist that I wanted to and she is going to set up a consultation to see if there is something else that we can be doing to help with pain management and almost all of my pain management cases we need to cotreat and this is the next best option pain level wise fluctuates but she is noticing more and more with each passing day that it is more manageable she does not have any new injury or any new changes to her medical history. Objectively today active range of motion is pretty stable she has full range of motion of the lumbosacral and cervical spine with mild irritation passive range of motion is the same palpation of the tissues reveals mild tissue hypertonicity bilateral paraspinals lumbar to cervical spine levator Scap upper trap and piriformis Motion palpation of the specific segments right C5 left C1 and left SI are fixated assessment she is doing very well with the conservative care in fact we have reached what I would call a milestone in terms of pain management this is a perfect time for a coconsultation with a pain management specialist prognosis at this point time is better than it has been. Today's treatment consisted of myofascial release in the flexion distraction table for 8 minutes posterior head traction with Scap stabilization exercise physeal ball 8 minutes trigger point dry needling for 5 minutes see objective tissue and manipulation using diversified to the objective segments moving forward I believe that with a consultation with a pain specialist we will have more information and be able to manage her pain with a higher proficiency rate.

Assessment

Subjective objective and plan can be found other subjective. Overall assessment with Victoria is good she does have a pain management consultation coming up I hope we receive a few more answers to why she is in repetitive pain other than the fact that this is a chronic induced symptom of post whiplash. related pain. Overall prognosis is to be determined at this point time on multiple factors continuation of care is necessary to control pain.

Electronically signed by:
Justin Merryman, DC
Signed on: 17 Mar 2021 03:16 PM

SOAP Note 1 Note

Viktoriya A Pulliam
DOB: 06 Dec 1989 (31 yo Female)

Visit Date: 31 Mar 2021

Subjective

Victoria presented today with primary cervicalgia and lumbosacral arthralgia 1 being radicular in symptomatology pain intensity has been decreasing over time

Objective

Severe hypertonicity left trap and scalene group segmental dysfunctions noticed left C5

Assessment

Overall good prognosis we are waiting on a pain consultation we are able to decrease the referral into her left arm with each treatment and depending on what she does outside is very determining on how long lasting the side effects are.

Plan

CMT diversified objective segments along with trigger point dry needling of the objective tissue and myofascial release of the spine on the flexion distraction table for 10 minutes.

Electronically signed by:
Justin Merryman, DC
Signed on: 31 Mar 2021 09:22 AM

SOAP Note 1 Note

Viktoriya A Pulliam
DOB: 06 Dec 1989 (31 yo Female)

Visit Date: 14 Apr 2021

Subjective

Victoria presented today with primary cervicalgia and lumbosacral arthralgia 1 being radicular in symptomatology pain intensity has been decreasing over time

Allergies

No allergy history has been documented for this patient.

Objective

Severe hypertonicity left trap and scalene group segmental dysfunctions noticed left C5.

Assessment

Victoria does have an appointment scheduled today with Dr. Mandel. But other than that she has been doing well with no new pain or injury. she is dealing with mostly secondary to her job but all stemming from this car wreck and now that we're working the full fascial planes she is feeling that she actually gets more free range of not only motion but just flexibility in general sleep has been doing better which contributes to her overall decrease in pain as well.

Plan

CMT diversified objective segments along with trigger point dry needling of the objective tissue and myofascial release of the spine on the flexion distraction table for 10 minutes.

Electronically signed by:
Justin Merryman, DC
Signed on: 14 Apr 2021 07:59 AM

SOAP Note 1 Note

Viktoriya A Pulliam
DOB: 06 Dec 1989 (31 yo Female)

Visit Date: 28 Apr 2021

Subjective

Victoria presented today with primary cervicalgia and lumbosacral arthralgia 1 being radicular in symptomatology pain intensity has been decreasing over time

Objective

multiple areas of spinal restriction. C5 right T1 left t2 left t6 left. hypertonicity of bi-lateral levator scapulae

Assessment

Victoria does have an appointment scheduled today with Dr. Mandel and will be doing the procedure today. But other than that she has been doing well with no new pain or injury. she is dealing with mostly secondary to her job but all stemming from this car wreck and now that we're working the full fascial planes she is feeling that she actually gets more free range of not only motion but just flexibility in general sleep has been doing better which contributes to her overall decrease in pain as well.

Plan

CMT diversified objective segments along with trigger point dry needling of the objective tissue and myofascial release of the spine on the flexion distraction table for 10 minutes.

Electronically signed by:
Justin Merryman, DC
Signed on: 28 Apr 2021 05:39 PM

Superbills

DATE OF VISIT	STATUS	PROVIDER	PATIENT	Dx	Procedure Codes
28 Apr 2021	Pending	Justin Merryman	Viktoriya A Pulliam	M43.8X7	98942 Chiropractic manipulative treatment (CMT); spinal, 5 regions
14 Apr 2021	Pending	Justin Merryman	Viktoriya A Pulliam	M43.8X7	98942 Chiropractic manipulative treatment (CMT); spinal, 5 regions
17 Mar 2021	Pending	Justin Merryman	Viktoriya A Pulliam	M43.8X7	98942 Chiropractic manipulative treatment (CMT); spinal, 5 regions
01 Mar 2021	Pending	Justin Merryman	Viktoriya A Pulliam	M43.8X7	98942 Chiropractic manipulative treatment (CMT); spinal, 5 regions
18 Feb 2021	Pending	Justin Merryman	Viktoriya A Pulliam	M43.8X7	98942 Chiropractic manipulative treatment (CMT); spinal, 5 regions
12 Feb 2021	Pending	Justin Merryman	Viktoriya A Pulliam	M43.8X7	98942 Chiropractic manipulative treatment (CMT); spinal, 5 regions
03 Feb 2021	Pending	Justin Merryman	Viktoriya A Pulliam	M43.8X7	98942 Chiropractic manipulative treatment (CMT); spinal, 5 regions
27 Jan 2021	Pending	Justin Merryman	Viktoriya A Pulliam	M43.8X7	98942 Chiropractic manipulative treatment (CMT); spinal, 5 regions
20 Jan 2021	Pending	Justin Merryman	Viktoriya A Pulliam	M43.8X7	98942 Chiropractic manipulative treatment (CMT); spinal, 5 regions
15 Jan 2021	Pending	Justin Merryman	Viktoriya A Pulliam	M43.8X7	98942 Chiropractic manipulative treatment (CMT); spinal, 5 regions
13 Jan 2021	Pending	Justin Merryman	Viktoriya A Pulliam	M43.8X7	98942 Chiropractic manipulative treatment (CMT); spinal, 5 regions
28 Dec 2020	Pending	Justin Merryman	Viktoriya A Pulliam	M43.8X7	98942 Chiropractic manipulative treatment (CMT); spinal, 5 regions
14 Dec 2020	Pending	Justin Merryman	Viktoriya A Pulliam	M43.8X7	98942 Chiropractic manipulative treatment (CMT); spinal, 5 regions
09 Dec 2020	Pending	Justin Merryman	Viktoriya A Pulliam	M43.8X7	98942 Chiropractic manipulative treatment (CMT); spinal, 5 regions
11 Nov 2020	Pending	Justin Merryman	Viktoriya A Pulliam	M43.8X7	99201 Office/op visit, new pt, 3 key components: prob focus hx; prob focus exam; strtfrwd med decision
06 Nov 2020	Pending	Justin Merryman	Viktoriya A Pulliam	M43.8X7	98942 Chiropractic manipulative treatment (CMT); spinal, 5 regions
30 Oct 2020	Pending	Justin Merryman	Viktoriya A Pulliam	M43.8X7	72100 Radiologic examination, spine, lumbosacral; 2 or 3 views
23 Oct 2020	Pending	Justin Merryman	Viktoriya A Pulliam	M43.8X7	72100 Radiologic examination, spine, lumbosacral; 2 or 3 views

VIKTORIYA A PULLIAM
DOB: 06 Dec 1989 (31 yo)

Date of Visit: 23 Oct 2020
Bill Provider: Justin Merryman
Service Location: Merryman Chiropractic
201 W Lakeway Rd Ste 211 , Gillette, WY, 827186341, United States of America

Code/Qty/Modifier	***CPT Description***	***(ICD10/ICD9) Code Rank/Description***
72100 (x1)	Radiologic examination, spine, lumbosacral; 2 or 3 views	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region
98942 (x1)	Chiropractic manipulative treatment (CMT); spinal, 5 regions	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region
97140 (x1)	Manual therapy techniques (eg, mobilization/ manipulation, manual lymphatic drainage, manual tractio...	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region
99203	Office or other outpatient visit for the evaluation and management of a new patient, which requires ...	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region

Billing Status: Pending

VIKTORIYA A PULLIAM
DOB: 06 Dec 1989 (31 yo)

Date of Visit: 30 Oct 2020
Bill Provider: Justin Merryman
Service Location: Merryman Chiropractic
201 W Lakeway Rd Ste 211 , Gillette, WY, 827186341, United States of America

Code/Qty/Modifier	***CPT Description***	***(ICD10/ICD9) Code Rank/Description***
72100 (x1)	Radiologic examination, spine, lumbosacral; 2 or 3 views	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region
98942 (x1)	Chiropractic manipulative treatment (CMT); spinal, 5 regions	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region
97140 (x1)	Manual therapy techniques (eg, mobilization/ manipulation, manual lymphatic drainage, manual tractio...	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region

Billing Status: Pending

VIKTORIYA A PULLIAM
DOB: 06 Dec 1989 (31 yo)

Date of Visit: 09 Dec 2020
Bill Provider: Justin Merryman
Service Location: Merryman Chiropractic
201 W Lakeway Rd Ste 211 , Gillette, WY, 827186341, United States of America

Code/Qty/Modifier	CPT Description	(ICD10/ICD9) Code Rank/Description
98942 (x1)	Chiropractic manipulative treatment (CMT); spinal, 5 regions	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region
97140 (x1)	Manual therapy techniques (eg, mobilization/ manipulation, manual lymphatic drainage, manual tractio...	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region

Billing Status: Pending

VIKTORIYA A PULLIAM
DOB: 06 Dec 1989 (31 yo)

Date of Visit: 11 Nov 2020
Bill Provider: Justin Merryman
Service Location: Merryman Chiropractic
201 W Lakeway Rd Ste 211 , Gillette, WY, 827186341, United States of America

Code/Qty/Modifier	CPT Description	(ICD10/ICD9) Code Rank/Description
99201	Office/op visit, new pt, 3 key components: prob focus hx; prob focus exam; strtfrwd med decision	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region

Billing Status: Pending

VIKTORIYA A PULLIAM
DOB: 06 Dec 1989 (31 yo)

Date of Visit: 06 Nov 2020
Bill Provider: Justin Merryman
Service Location: Merryman Chiropractic
201 W Lakeway Rd Ste 211 , Gillette, WY, 827186341, United States of America

Code/Qty/Modifier	CPT Description	(ICD10/ICD9) Code Rank/Description
98942 (x1)	Chiropractic manipulative treatment (CMT); spinal, 5 regions	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region
97140 (x1)	Manual therapy techniques (eg, mobilization/ manipulation, manual lymphatic drainage, manual tractio...	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region

Billing Status: Pending

VIKTORIYA A PULLIAM
DOB: 06 Dec 1989 (31 yo)

Date of Visit: 28 Dec 2020
Bill Provider: Justin Merryman
Service Location: Merryman Chiropractic
201 W Lakeway Rd Ste 211 , Gillette, WY, 827186341, United States of America

Code/Qty/Modifier	*CPT Description*	*(ICD10/ICD9) Code Rank/Description*
98942 (x1)	Chiropractic manipulative treatment (CMT); spinal, 5 regions	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region
97140 (x1)	Manual therapy techniques (eg, mobilization/ manipulation, manual lymphatic drainage, manual tractio...	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region

Billing Status: Pending

VIKTORIYA A PULLIAM
DOB: 06 Dec 1989 (31 yo)

Date of Visit: 14 Dec 2020
Bill Provider: Justin Merryman
Service Location: Merryman Chiropractic
201 W Lakeway Rd Ste 211 , Gillette, WY, 827186341, United States of America

Code/Qty/Modifier	*CPT Description*	*(ICD10/ICD9) Code Rank/Description*
98942 (x1)	Chiropractic manipulative treatment (CMT); spinal, 5 regions	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region
97140 (x1)	Manual therapy techniques (eg, mobilization/ manipulation, manual lymphatic drainage, manual tractio...	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region

Billing Status: Pending

VIKTORIYA A PULLIAM
DOB: 06 Dec 1989 (31 yo)

Date of Visit: 20 Jan 2021
Bill Provider: Justin Merryman
Service Location: Merryman Chiropractic
201 W Lakeway Rd Ste 211 , Gillette, WY, 827186341, United States of America

Code/Qty/Modifier	*CPT Description*	*(ICD10/ICD9) Code Rank/Description*
98942 (x1)	Chiropractic manipulative treatment (CMT); spinal, 5 regions	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region
97110 (x1)	Therapeutic procedure, 1 or more areas, each 15 minutes; therapeutic exercises to develop strength a...	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region

Billing Status: Pending

VIKTORIYA A PULLIAM
DOB: 06 Dec 1989 (31 yo)

Date of Visit: 15 Jan 2021
Bill Provider: Justin Merryman
Service Location: Merryman Chiropractic
201 W Lakeway Rd Ste 211 , Gillette, WY, 827186341, United States of America

Code/Qty/Modifier	*CPT Description*	*(ICD10/ICD9) Code Rank/Description*
98942 (x1)	Chiropractic manipulative treatment (CMT); spinal, 5 regions	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region

Billing Status: Pending

VIKTORIYA A PULLIAM
DOB: 06 Dec 1989 (31 yo)

Date of Visit: 13 Jan 2021
Bill Provider: Justin Merryman
Service Location: Merryman Chiropractic
201 W Lakeway Rd Ste 211 , Gillette, WY, 827186341, United States of America

Code/Qty/Modifier	*CPT Description*	*(ICD10/ICD9) Code Rank/Description*
98942 (x1)	Chiropractic manipulative treatment (CMT); spinal, 5 regions	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region

Billing Status: Pending

VIKTORIYA A PULLIAM
DOB: 06 Dec 1989 (31 yo)

Date of Visit: 03 Feb 2021
Bill Provider: Justin Merryman
Service Location: Merryman Chiropractic
201 W Lakeway Rd Ste 211 , Gillette, WY, 827186341, United States of America

Code/Qty/Modifier	CPT Description	(ICD10/ICD9) Code Rank/Description
98942 (x1)	Chiropractic manipulative treatment (CMT); spinal, 5 regions	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region
97110 (x1)	Therapeutic procedure, 1 or more areas, each 15 minutes; therapeutic exercises to develop strength a...	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region

Billing Status: Pending

VIKTORIYA A PULLIAM
DOB: 06 Dec 1989 (31 yo)

Date of Visit: 27 Jan 2021
Bill Provider: Justin Merryman
Service Location: Merryman Chiropractic
201 W Lakeway Rd Ste 211 , Gillette, WY, 827186341, United States of America

Code/Qty/Modifier	CPT Description	(ICD10/ICD9) Code Rank/Description
98942 (x1)	Chiropractic manipulative treatment (CMT); spinal, 5 regions	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region
97110 (x1)	Therapeutic procedure, 1 or more areas, each 15 minutes; therapeutic exercises to develop strength a...	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region

Billing Status: Pending

VIKTORIYA A PULLIAM
DOB: 06 Dec 1989 (31 yo)

Date of Visit: 18 Feb 2021
Bill Provider: Justin Merryman
Service Location: Merryman Chiropractic
201 W Lakeway Rd Ste 211 , Gillette, WY, 827186341, United States of America

Code/Qty/Modifier	*CPT Description*	*(ICD10/ICD9) Code Rank/Description*
98942 (x1)	Chiropractic manipulative treatment (CMT); spinal, 5 regions	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region
97110 (x1)	Therapeutic procedure, 1 or more areas, each 15 minutes; therapeutic exercises to develop strength a...	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region

Billing Status: Pending

VIKTORIYA A PULLIAM
DOB: 06 Dec 1989 (31 yo)

Date of Visit: 12 Feb 2021
Bill Provider: Justin Merryman
Service Location: Merryman Chiropractic
201 W Lakeway Rd Ste 211 , Gillette, WY, 827186341, United States of America

Code/Qty/Modifier	*CPT Description*	*(ICD10/ICD9) Code Rank/Description*
98942 (x1)	Chiropractic manipulative treatment (CMT); spinal, 5 regions	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region
97110 (x1)	Therapeutic procedure, 1 or more areas, each 15 minutes; therapeutic exercises to develop strength a...	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region

Billing Status: Pending

VIKTORIYA A PULLIAM
DOB: 06 Dec 1989 (31 yo)

Date of Visit: 17 Mar 2021
Bill Provider: Justin Merryman
Service Location: Merryman Chiropractic
201 W Lakeway Rd Ste 211 , Gillette, WY, 827186341, United States of America

Code/Qty/Modifier	*CPT Description*	*(ICD10/ICD9) Code Rank/Description*
98942 (x1)	Chiropractic manipulative treatment (CMT); spinal, 5 regions	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region
97110 (x1)	Therapeutic procedure, 1 or more areas, each 15 minutes; therapeutic exercises to develop strength a...	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region

Billing Status: Pending

VIKTORIYA A PULLIAM
DOB: 06 Dec 1989 (31 yo)

Date of Visit: 01 Mar 2021
Bill Provider: Justin Merryman
Service Location: Merryman Chiropractic
201 W Lakeway Rd Ste 211 , Gillette, WY, 827186341, United States of America

Code/Qty/Modifier	*CPT Description*	*(ICD10/ICD9) Code Rank/Description*
98942 (x1)	Chiropractic manipulative treatment (CMT); spinal, 5 regions	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region
97110 (x1)	Therapeutic procedure, 1 or more areas, each 15 minutes; therapeutic exercises to develop strength a...	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region

Billing Status: Pending

VIKTORIYA A PULLIAM
DOB: 06 Dec 1989 (31 yo)

Date of Visit: 28 Apr 2021
Bill Provider: Justin Merryman
Service Location: Merryman Chiropractic
201 W Lakeway Rd Ste 211 , Gillette, WY, 827186341, United States of America

Code/Qty/Modifier	*CPT Description*	*(ICD10/ICD9) Code Rank/Description*
98942 (x1)	Chiropractic manipulative treatment (CMT); spinal, 5 regions	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region
97110 (x1)	Therapeutic procedure, 1 or more areas, each 15 minutes; therapeutic exercises to develop strength a...	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region

Billing Status: Pending

VIKTORIYA A PULLIAM
DOB: 06 Dec 1989 (31 yo)

Date of Visit: 14 Apr 2021
Bill Provider: Justin Merryman
Service Location: Merryman Chiropractic
201 W Lakeway Rd Ste 211 , Gillette, WY, 827186341, United States of America

Code/Qty/Modifier	*CPT Description*	*(ICD10/ICD9) Code Rank/Description*
98942 (x1)	Chiropractic manipulative treatment (CMT); spinal, 5 regions	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region
97110 (x1)	Therapeutic procedure, 1 or more areas, each 15 minutes; therapeutic exercises to develop strength a...	1. (M43.8X7) Other specified deforming dorsopathies, lumbosacral region 2. (M99.05) Segmental and somatic dysfunction of pelvic region 3. (M99.01) Segmental and somatic dysfunction of cervical region 4. (M99.02) Segmental and somatic dysfunction of thoracic region

Billing Status: Pending

Tasks / Messages

PRIORITIES

REMINDERS

- No Items

MESSAGES

SUBJECT	DATE/TIME

CONFIDENTIAL Fax

FROM:

JOHN L MANSELL, MD
3100 W Lakeway Rd, Gillette, WY 82718-6373
Phone: (307) 696-2996
Fax: (307) 670-8250

TO:

JUSTIN MERRYMAN DC
201 W LAKEWAY RD, GILLETTE, WY 82718
Phone: (307) 217-2323
Fax: (307) 228-2184

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A WYOMING PAIN CLINIC, P.C. • 3100 W Lakeway Rd, Gillette WY 82718-6373
PULLIAM, VIKTORIYA (id #6945, dob: 12/06/1989)
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[ID:318480-H-11359]

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Gillette, WY 82718-6373
Phone: (307) 696-2996, Fax: (307) 670-8250

Date: 04/14/2021

RE: Viktoriya Pulliam, DOB: 12/06/1989

Dear Dr. Merryman,

I would like to thank you for referring Viktoriya Pulliam who underwent bilateral L3 to S1 diagnostic medial branch blocks today.

The procedure went smoothly and she tolerated it well, reporting her pain dropped from a "3" to a "1" or "75%" better. She was sent home with a three-day pain diary as well.

Once again, thank you for allowing me to participate in the care of this patient.

Sincerely,

Electronically Signed by: JOHN L MANSELL, MD

Procedure Documentation

Lumbar Medial Branch Blocks:
Bilateral L3 to S1 Lumbar Medial Branch Blocks

Risks of lumbar medial branch blocks were reviewed with the patient including bleeding, infection, nerve damage, spinal cord damage, paraplegic, quadriplegic, permanent incontinence and death. The patient is taking no anticoagulant medications and has a review of systems negative for coagulopathic symptoms including bleeding, infection, nerve damage, spinal cord damage, paraplegic, quadriplegic, permanent incontinence and death. They are afebrile with no recent infectious disease symptoms. Informed consent was obtained.

In a prone position on the fluoroscopy table after a timeout was performed the transverse processes from L3

to S1 bilaterally were visualized fluoroscopically and after prepping and draping in the usual sterile fashion through topical cold spray and an acupuncture needle insertion technique a 25gauge 3.5" needle was placed down to the superior aspect of the medial third of each of these transverse processes bilaterally at these levels. Following a negative aspirate on each needle 1 mL of Omnipaque 200 contrast was injected through each needle yielding a supra-cortical pattern of spread. Following another negative aspirate 1 mL of 0.75% bupivacaine was injected through each needle displacing the previously noted dye patterns. The needles were withdrawn intact. Band-Aids were placed on the sites.

The patient was returned to the post-procedure observation area where after one hour they reported their pain dropped from an "3" to a "1" or "75%" better. She was sent home with a three-day pain diary as well.

A list of side effects, complications and their symptoms were reviewed with them and they were instructed to contact us should any of these give them concern. The transient nature of diagnostic blocks was explained to them as well and they were discharged without incident. They tolerated the procedure well.

A WYOMING PAIN CLINIC, P.C. • 3100 W Lakeway Rd, Gillette WY 82718-6373

PULLIAM, VIKTORIYA (id #6945, dob: 12/06/1989)

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John Mansell, MD
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3100 W Lakeway Rd Suite 3
Gillette, WY 82718-6373
Phone: (307) 696-2996, Fax: (307) 670-8250

Date: 04/28/2021

RE: Viktoriya Pulliam, DOB: 12/06/1989

Dear Dr. Merryman,

I would like to thank you for referring Viktoriya Pulliam who underwent bilateral L3 to S1 radiofrequency ablation of medial branch nerves for her axial back pain today.

The procedure went smoothly and she tolerated it well. For her agi group we tend to see about 8 to 12 months of rlief, during which time we encourage exercise to optimize her back mechanics so when the nerve sregenerate she might not need me to repeat the procedure.

Once again, thank you for allowing me to participate in the care of this patient.

Sincerely,

Electronically Signed by: JOHN L MANSELL, MD

Procedure Documentation

Lumbar Radiofrequency Ablations of Medial Branch Nerves:
Bilateral L3 to S1 Radiofrequency Ablations of Medial Branch Nerves

Risks of this were reviewed with her including bleeding, infection, nerve damage, spinal cord damage, paraplegia, quadriplegia, permanent incontinence and death. She is taking no blood thinner medications and has a review of systems negative for coagulopathic symptoms. She is afebrile with no recent infectious disease symptoms. Informed consent was obtained.
In a prone position on the fluoroscopy table after a timeout was observed her back was prepped widely with Betadine and draped in the usual sterile fashion. Using a C-arm telescopic unit and an AP view the transverse processes bilaterally at L3, L4 and L5 as well as the sacral alae were visualized and through topical cold spray and an acupuncture needle insertion technique 18gauge radiofrequency needles were placed on the superior aspect of the medial third of each of these locations at these levels bilaterally and incremental moves were made until successful stimulation was achieved at 50 Hz at less than .5 V with negative motor

stimulation at 2 Hz at triple the sensory threshold. After this was accomplished 2.5 mL of 0.75% bupivacaine with 4 mg/mL of Depo-Medrol admixed was injected to each needle and we waited five minutes before performing ablations at 80°C for 75 seconds. Two 120° clockwise turns were performed within ablation after each turn for a total of three ablations at each level. All needles were withdrawn intact. Band-Aids were placed on the sites.

She was returned to the postprocedure observation area where she appeared to tolerate the procedure well. A list of side effects, complications and their symptoms were reviewed with her and she was instructed to contact us should any of these give them concern. She tolerated the procedure well and was discharged without incident.

He was returned to the postprocedure observation area where he appeared to tolerate the procedure well. A list of side effects, complications and their symptoms were reviewed with him and he was instructed to contact us should any of these give him concern. The delayed onset of steroid effect with transit exacerbation of symptoms was explained to him as well and he was discharged without incident.

New Spine Chiropractic
12766 SE Stark St., Plaza 125 (C)
Portland, OR 97233
P: 503 232 2933 F: 971 271 7054

Viktoriya Pulliam
Patient ID: 95887598 DOB: 12/06/1989 Sex: F Account No.:
Encounter ID: 216922683 Encounter Date: 02/12/2020
Encounter Type: Office Visit

SUBJECTIVE:

Chief Complaint: Vicky reports that she was in a second accident yesterday (2/11/2020.) She states that the second accident has increased her symptoms by approximately 65% and has reaggravated her previous injuries. Please see included intake questionaire for details regarding second accident that occurred on 2/11/2020. 65% of patients current symptoms/injuries are attributable to the second accident and 35% of patient symptoms/injuries are attributable to the patients first accident which occurred on 11/21/19.

OBJECTIVE:

Objective Notes: Static palpation reveals: +2/4 Tight and Tender suboccipitals Sternocleidomastoids, levator scapula, upper traps. Rhomboids, scalenes, cervical thoracic lumbar paraspinals, quaratus lumborum, and elft wrist extensors. Segmental Analysis reveals: restrictions at C2-4R/LRR, T4-8 ext wirh R ribs, L4-S1R/LRR, R SI ext. Cervical AROM: flexion 36/45. extension 38/55. L Lateral Flexion 32/45. R Lateral Flexion 32/45. L Rotation 58/70. R Rotation 60/70. Thoraco/lumbar AROM: Flexion 64/90. Extension 22/30. L Lat Felx 16/20. R Lat Flex 16/20. L Rotation 22/30. R Rotation 20/30. Resisted Neck MSL Test cuases BL Cx and UT pain BL, Max Cx Compression causes CT tenderness BL, Left Shoulder Depression causes: tender and tight L UT and Cx spine, Right shoulder depression causes: tight and tender right UT and Cx spine. Kemps test causes: BL TLS tenderness, DTRs WNL U/LE. ight touch NWL BL in U/LEs.

ASSESSMENT:

Diagnosis: ICD-10 Codes:

1)S060X0D; Concussion without loss of consciousness, subsequent encounter
2)S138XXA; Sprain of joints and ligaments of other parts of neck, initial encounter
3)S233XXA; Sprain of ligaments of thoracic spine, initial encounter
4)S335XXA; Sprain of ligaments of lumbar spine, initial encounter
5)R51; Headache
6)M609; Myositis, unspecified
7)M62838; Other muscle spasm
8)M9907; Segmental and somatic dysfunction of upper extremity
9)M9901; Segmental and somatic dysfunction of cervical region
10)M9902; Segmental and somatic dysfunction of thoracic region
11)M9903; Segmental and somatic dysfunction of lumbar region
12)M9904; Segmental and somatic dysfunction of sacral region

Assessments: ICD-10 Assessments:

Patients prognosis: await response to care after second accident.

PLAN:

Procedures:

1) 98941; Sp. Manipulation 3 - 4
2) 97124; Massage
3) 99212; Est. Pt. Prob Focused

Procedure Notes: Re evaluation performed by Dr. gentry, due to second collision. Light CMT (manual to Cx, activator to Tx, drop to LX.) to CTLS spine, see detailed listing above, Massage therapy provided by LMT see notes below:

Pt is feeling increase tenderness and tightness in neck, shoulders, and back after secind accident. I found HT in BL rhombs, CTL PS, QLs, BL UTs, subocos, R sclenes and traps, and BL glutes, Myofascial and med tissue, applied to muscles listed above. 4 units 902am-957am, Brannigan Roeber LMT 23721

Care Plan:

New Spine Chiropractic
12766 SE Stark St., Plaza 125 (C)
Portland, OR 97233
P: 503 232 2933 F: 971 271 7054

Viktoriya Pulliam
Patient ID: 95887598 DOB: 12/06/1989 Sex: F Account No.:
Encounter ID: 216922683 Encounter Date: 02/12/2020
Encounter Type: Office Visit

Treatment is directed to return the patient to pre-injury status. Post Treatment Symptoms/Signs: Pt. reports a decrease in stiffness and soreness. After evaluation of symptoms and injuries from second accident it is estimated that 65% of patients current injuries and Sx are due to her second collision and 35% are due to her first collision. Treatment will be 2x/week for 4-6 weeks then a re exam will be performed at that time.

Patient Instructions: Patient responded well to treatment today without complication. Pt advised to do Cx AROM stretches and exercises to help decrease disc pressure. After evaluation of symptoms and injuries from second accident it is estimated that 65% of patients current injuries and Sx are due to her second collision and 35% are due to her first collision

[Electronically Signed] - Date: 2/24/2020 10:45:46 AM

[Provider]: Solon Gentry

A WYOMING PAIN CLINIC, P.C. • 3100 W Lakeway Rd, Gillette WY 82718-6373
PULLIAM, VIKTORIYA (id #6945, dob: 12/06/1989)
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Phone: (307) 696-2996, Fax: (307) 670-8250

Date: 04/12/2021

RE: Viktoriya Pulliam, DOB: 12/06/1989

Dear Dr. Merryman,

I would like to thank you for referring Viktoriya Pulliam to our practice for consultation and evaluation of low back pain, on 04/12/2021.

Palpation over the L3, L4 and L5 spinous processes as well as lumbar extension were the most provocative exam maneuver and caused concordant pain. For this reason I recommended a trial of medial branch blocks at these levels.

I have enclosed a copy of the office evaluation for your records. Once again, thank you for allowing me to participate in the care of this patient.

Sincerely,

Electronically Signed by: JOHN L MANSELL, MD

Encounter Reason/Date

low back pain
04/12/2021 - 09:30AM - WPI-Gillette

Problems:

Reviewed Problems
No known problems

Allergies:

Reviewed Allergies
PENICILLINS

Medications:

Reviewed Medications

escitalopram 20 mg tablet TAKE 1 TABLET BY MOUTH EVERY DAY 03/12/21 filled

Family History:

Reviewed Family History

Unspecified Relation	- Family history of alcoholism - Grandfather - Diabetes mellitus - Grandmother - Family history of cancer - Grandmother- ovarian , brain ca
Brother	- Family history of substance abuse - Illegal Drug Abuse
Father	- Heart disease - Hypertensive disorder

Social History:

Reviewed Social History
Education and Occupation
Are you currently employed?: Yes
Substance Use
Do you or have you ever smoked tobacco?: Former smoker
Do you or have you ever used smokeless tobacco?: Never used smokeless tobacco
How much tobacco do you chew?: none
What was the date of your most recent tobacco screening?: 04/14/2021
Do you use any illicit or recreational drugs?: No
What is your level of caffeine consumption?: Moderate
Public Health and Travel
Have you been to an area known to be high risk for COVID-19?: No
Gender Identity and LGBTQ Identity
Gender identity: Choose not to disclose
Assigned sex at birth: Choose not to disclose
Sexual orientation: Choose not to disclose

Surgical History

Reviewed Surgical History

Additional History

None recorded

History of Present Illness:

This is a pleasant 31-year-old lady with a history of back pain that dates back to two motor vehicle accidents in November 2019 and February 2020. In the first accident she was struck from the driver side while restrained and in February 2020 she was struck on the passenger side. Since these accidents she has felt that her back pain has progressively gotten worse.

She describes a 3 inch high area of pain that spans her back with about 70% on the left side and 30% on the right which may also refer down the left lateral thigh in a 1 inch wide stripe to her knee.

Overall she apportions the pain is 90% in her back and 10% in her left leg.

She also reports pain over the lower thoracic spine that will also go down into her back on a daily basis with more severe episodes happening on average twice a week.

She describes the pain is achy with electric sensation as well as pressure, throbbing, horizontal and vertical pulling

and a sense of tightness in her back.

In the last month the pain is been as severe as 8/10 and as low as 0/10.

When she gets up in the morning the pain tends to be worse for about an hour and by noon it is typically more severe. It awakens her every night.

Exacerbating factors include standing more than 5 to 10 minutes, lying in any static position more than 30 minutes, rolling over in her sleep which will awaken her with pain, stairs, straightening up at the waist which is more painful than bending forward, lifting, driving or sitting more than 10 minutes and light housework which requires frequent breaks.

Mitigating factors include lying down on her sides, leaning on other objects to take the weight off of her back or topical heat.

Associated symptoms include tingling in her legs the long drives, muscle spasms causing hip flexion as well as clicking and fine crepitance in her back. She denies any weakness or unintentional weight loss or changes in bladder or rectal control. She does say that the last year she has gained about 25 pounds.

Her most recent Pap smear was this year. She underwent an ectopic surgery in 2019 but does not feel this affected her back pain. She has taken Advil in this is helped a little bit.

Her opioid risk tool score is for or medium for a family history of alcohol and illegal drug abuse and her youth. Her urine drug screen is unremarkable.

I spent 45 minutes with this patient which consisted of chart review, patient interview and examination, documentation and coordination of care all on the same day. Time is the basis of this E & M CPT code.

Review of Systems:

Constitutional:
Constitutional: no fever or night sweats.

Eyes:
Eyes: **vision change**.

ENMT:
Nose: no frequent nosebleeds. Mouth/Throat: no bleeding gums.

Cardiovascular:
Cardiovascular: no palpitations.

Gastrointestinal:
Gastrointestinal: **nausea**, **vomiting**, and **constipation**.

Genitourinary:
Genitourinary: no incontinence or hematuria.

Musculoskeletal:
Musculoskeletal: **back pain**.

Integumentary:
Skin: no jaundice or rashes.

Neurologic:
Neurologic: no seizures or loss of consciousness and **dizziness** and **frequent or severe headaches**.

Psychiatric:
Psych: no hallucinations and **suicidal thoughts**.

Hematologic/Lymphatic:
Hematologic/Lymphatic no bruising or excessive bleeding.

Allergic/Immunologic:
Allergy/Immunologic: no itching or hives.

Physical Exam

Patient is a 31-year-old female.

Chaperone:
Chaperone: offered and declined.

Constitutional:
General Appearance: healthy-appearing, well-nourished, and well-developed. Level of Distress: NAD.

Psychiatric:
Insight: good judgement. Mental Status: normal mood and affect and active and alert. Orientation: to time, place, and person. Memory: recent memory normal and remote memory normal.

Head:
Head: normocephalic and atraumatic.

Eyes:
Lids and Conjunctivae: non-injected and no discharge. Pupils: PERRLA. EOM: EOMI. Sclerae: non-icteric.

ENMT:
Nose: no lesions on external nose or nasal discharge. Lips, Teeth, and Gums: no bleeding gums. Oropharynx: moist mucous membranes.

Lungs:
Respiratory effort: no dyspnea.

Cardiovascular:
Pulses including femoral / pedal: radial right +2 and +1, radial left +2, posterior tibialis left: 1+ and right: 1+, and dorsalis pedis left: 1+.

Abdomen:
Bowel Sounds: normal. Inspection and Palpation: soft, non-distended, and no tenderness.

Musculoskeletal::
Joints, Bones, and Muscles: no pain with log roll: left or right, no leg length discrepancy, and nontender to palpation: left greater trochanter and palpation: right greater trochanter. Extremities: no edema.

Neurologic:
Cranial Nerves: grossly intact. Sensation: grossly intact. Coordination and Cerebellum: no tremor. Femoral Nerve Stretch: femoral nerve traction test negative (**bilaterally**). Lumbar / Lumbosacral Spine negative left straight leg raise and right straight leg raise. Hoffmans Lumbar Reflex Integrity Left foot Hoffman's normal (-) and Right foot Hoffman's normal (-). ATHENA Cervical Spine Strength **Symmetric and normal strength deltoids, triceps, biceps, wrist flexion and extension and grip, symmetric and low normal strength with hip flexion, extension, abduction and adduction, knee flexion and extension as well as dorsiflexion plantarflexion at the ankles.**.

Skin:
Inspection and palpation: good turgor and no jaundice.

Back:
Thoracolumbar Appearance: **normal lumbar lordosis with increased pain with lumbar extension and lumbar axial rotation, tender over the L3, L4 and L5 spinous processes but not the PSIS bilaterally, normal lumbar muscle tone with no palpable trigger points.**.

Procedure Documentation

None recorded

Assessment/Plan

Palpation over the L3, L4 and L5 spinous processes as well as lumbar extension were the most provocative exam maneuver and caused concordant pain. For this reason I recommended a trial of medial branch blocks at these levels.

1. Chronic pain syndrome
G89.4: Chronic pain syndrome
- DRUG SCREEN, URINE -
  Specimen source: Urine

2. Lumbosacral spondylosis without myelopathy
M47.817: Spondylosis without myelopathy or radiculopathy, lumbosacral region

DRUG SCREEN, URINE
- Results:
  - **THC: negative**
  - **Temperature: 94 degrees**
  - **OPI: negative**
  - **AMP: negative**
  - **MET: negative**
  - **PCP: negative**
  - **BZO: negative**
  - **BAR: negative**

- **MDMA: negative**
- **MTD: negative**
- **OXY: negative**
- **TCA: negative**
- **BUP: negative**
- **EDDP: negative**
- **COC: negative**

Return to Office
None recorded

Open Advanced MRI™

A subdivision of Precision Imaging & Interventional Pain, Inc

MRI- SPINE- CERVICAL

Patient Name: VIKTORIYA PULLIAM
Date of Birth: 12/06/1989
Referring Physician: SOLON GENTRY DC
Date of Service: 01/22/2020
Cervical

CLINICAL INDICATION: At intake patient reports and/or draws on the body diagram neck pain with radicular-type symptoms. Specifically patient indicates pain radiation into the right shoulder and right upper extremity numbness. Symptoms described as a subsequent to involvement in MVA on 11.21.2019. Patient is claustrophobic, reason for open MRI system request. Referred by the division of chiropractic medicine.

TECHNIQUE: MR imaging of the cervical spine is obtained on the open MRI system. This to include sagittal T1 FSE, sagittal T2 FSE, axial T2 FSE and coronal T1 FSE. 5 cc Multihance is injected intravenously via peripheral access through the right antecubital fossa, this without an air observed complications. Postcontrast T1 FSE fat sat imaging is subsequently obtained. Patient's BMI is 21 based on the NIH calculator. In addition the entire high spatial resolution, enhance fluid signal, gradient echo-type imaging of the central canal, reconstructed in MIPsagittalpbl sequence, standard orthogonal and curved MPR projections, the latter 3-D reconstructions on injected/enhancement performed by the radiologist. Comparison: None similar.

FINDINGS: Reduced cervical lordosis and slight leftward neck-head tilt, nonspecific.
Superimposed slight/1-2 mm displacement in the direction of retrolisthesis at C5-6, key image 1.
No Modic type I reaction or subchondral marrow edema within the articular facets.
Suggested slight diastases of the left C5-6 facet joint, key image 2.
Short pedicle, congenitally narrow bony spinal canal configuration.

The general cord caliber is in the normal range.
Discogenic mass-effect on cord is detailed in the level by level analysis below.
This noted there is increased T2 signal within the ventral left parasagittal cord,
- this 1.5-2 mm in extent, key images 3-4.
Right cerebellar ectopia without mass-effect on the adjacent brainstem, key image 5.
There is no syrinx.

Posterior epidural fat at C7-T1 is size permissible for image guided procedures.
The vertebral arteries enter the foramina transversarium at C6; the right is dominant.
No incidental/ominous soft tissue abnormality on this limited assessment of the neck.

C1-2: Unremarkable.
C2-3: No annular injury or disc herniation. No facet joint effusion or strain pattern.
C3-4: No annular injury or disc herniation. No facet joint effusion or strain pattern.
C4-5: No annular injury or disc herniation. No facet joint effusion or strain pattern.

C5-6: Increased T2 signal within the posterior paracentral annulus, key image 6.
Superimposed posterior paracentral disc herniation, 9-10 mm/base and 3 mm/radial extent, key images 6-7.
Disc flattens the ventral cord surface and encroaches the ventral C6 nerve rootlets right more than left.
Rim peri-annular hyperemia along the herniated disc, key image 7.
Mildly thickened ligamentum flavum that effaces the dorsal CSF space.
The mid sagittal central canal measures 5 mm, substantially narrowed.
No significant neural foramina narrowing.
No facet joint effusions or intra-articular/capsular hyperemia.

1026 NW Stone Ave., Gresham, OR 97030, Phone 503-469-1674 Fax 503-489-1678
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www.OpenAdvancedMRINW.com

RE: VIKTORIYA PULLIAM ACCNT#: 25221 DOS: 01/22/2020

C6-7: No annular injury or disc herniation. No facet joint effusions. Borderline right facet joint hyperemia
C7-T1: No annular injury or disc herniation. No facet joint effusion or strain pattern.

IMPRESSION:

1. Subluxation of C5-6 cervical vertebra, initial encounter (S13.160A)

Singular level alignment deformity indicative of disc/posterior longitudinal ligament injury.

2. Traumatic rupture of cervical intervertebral disc, initial encounter (S13.0XXA)
Traumatic spondylopathy, cervical region (M48.32)
Cervical disc disease C5-6 (M50.122)

Annular injury/tear and superimposed disc herniation at C5-6.
This results in mass-effect on the ventral cord also on the ventral C6 nerve rootlets right side worse.
Associated active peri-annular granulation compatible with a subacute temporality.
In the context of recent MVA, this more probable than not posttraumatic in etiology.

3. Suggested subluxation of the left C5-6 facet joint.
Ongoing inflammation indicative of active strain/likely compensatory of the right C6-7 facet joint.

Recommendation: Patient may return to our clinic for consultation/consideration of image guided pain therapy.
This performed at the discretion of the referring clinician in addition to ongoing conservative therapy.
This considered as most appropriate when conservative therapy is not conclusive and surgery not yet contemplated.
If recommended by the provider, therapy to be initiated at C7-T1 interlaminar ESI.
Enhanced response to conservative + minimally invasive therapy may further substantiate the medical necessity for surgery.

Note to Provider: The medical imaging information, to a large extent, may be better communicated when the key images, as reported above, are reviewed by the provider, other consultants and/or patients. Please let me know, at the email address below my signature, if you need to review the key images, the PACS system(s) you have access to and if further clarification required for the imaging report.

Thank you for engaging us to contribute to the care of your patient. Always let us know if we could have done better.

VR-Dragon Medical dictation integrated with the RMR-PACS system. If voice typographical errors are suspected whereby these may inadvertently affect in the intended clinical information of this report, please kindly contact the radiologist for prompt clarification and correction.
Electronically Signed- 01/22/20 9:22

1/22/2020 12:40 PM ALBC Page 3 of 3

RE: VIKTORIYA PULLIAM ACCNT#: 25221 DOS: 01/22/2020

Anthony E. Larhs, MD
alarhs@ozmrinw.com
Cell: 206-877-3469
If we are unable to take your call or text,
we will respond to your message promptly.

date of accident 11/21/19 #

date of accident 2/11/20 #

Merryman Spinal and Sports

Itemization of Charges

For Posting Date July 8, 2021

Clinic: Merryman Spinal and Sports

Address:

Phone:

Tax ID: 814086398

WCAB:

Employer:

Patient #: 10283

Patient: Viktoriya Pulliam

Date of Injury:

Insurance #1:

Adjuster:

Group Number

Policy Number:

Insurance #2:

Adjuster:

Group Number:

Policy Number:

Visit #	Service Date	Provider Name	Procedure Description	Code	Charges	Adjust	Payments	Balance
17737	10/23/2020	Justin Merryman, DC	Radiologic examination, sp...	72100	65.00	-		-
17737	10/23/2020	Justin Merryman, DC	Manual therapy techniques ...	97140	48.00	-		-
17737	10/23/2020	Justin Merryman, DC	Chiropractic manipulative ...	98942	51.75	-		-
17737	10/23/2020	Justin Merryman, DC	Office or other outpatient...	99203	156.00	-		-
19053	10/30/2020	Justin Merryman, DC	Radiologic examination, sp...	72100	65.00	-		-
19053	10/30/2020	Justin Merryman, DC	Manual therapy techniques ...	97140	48.00	-		-
19053	10/30/2020	Justin Merryman, DC	Chiropractic manipulative ...	98942	51.75	-		-
20562	11/06/2020	Justin Merryman, DC	Manual therapy techniques ...	97140	48.00	-		-
20562	11/06/2020	Justin Merryman, DC	Chiropractic manipulative ...	98942	51.75	-		-
21696	11/11/2020	Justin Merryman, DC	OFFICE OUTPATIENT NEW 10 M...	99201	34.50	-		-
29016	12/09/2020	Justin Merryman, DC	Manual therapy techniques ...	97140	48.00	-		-
29016	12/09/2020	Justin Merryman, DC	Chiropractic manipulative ...	98942	51.75	-		-
30441	12/14/2020	Justin Merryman, DC	Manual therapy techniques ...	97140	48.00	-		-
30441	12/14/2020	Justin Merryman, DC	Chiropractic manipulative ...	98942	51.75	-		-
33190	12/28/2020	Justin Merryman, DC	Manual therapy techniques ...	97140	48.00	-		-
33190	12/28/2020	Justin Merryman, DC	Chiropractic manipulative ...	98942	51.75	-		-
36939	01/13/2021	Justin Merryman, DC	Chiropractic manipulative ...	98942	51.75	-		-
37359	01/15/2021	Justin Merryman, DC	Chiropractic manipulative ...	98942	51.75	-		-
38054	01/20/2021	Justin Merryman, DC	Therapeutic procedure, 1 o...	97110	23.00	-		-
38054	01/20/2021	Justin Merryman, DC	Chiropractic manipulative ...	98942	51.75	-		-
40405	01/27/2021	Justin Merryman, DC	Therapeutic procedure, 1 o...	97110	23.00	-		-
40405	01/27/2021	Justin Merryman, DC	Chiropractic manipulative ...	98942	51.75	-		-
42516	02/03/2021	Justin Merryman, DC	Therapeutic procedure, 1 o...	97110	23.00	-		-

Merryman Spinal and Sports

Itemization of Charges

For Posting Date July 8, 2021

42516	02/03/2021	Justin Merryman, DC	Chiropractic manipulative ...	98942	51.75	-		-
45248	02/12/2021	Justin Merryman, DC	Therapeutic procedure, 1 o...	97110	23.00	-		-
45248	02/12/2021	Justin Merryman, DC	Chiropractic manipulative ...	98942	51.75	-	)	-
47075	02/18/2021	Justin Merryman, DC	Therapeutic procedure, 1 o...	97110	23.00	-	)	-
47075	02/18/2021	Justin Merryman, DC	Chiropractic manipulative ...	98942	51.75	-		-
49457	03/01/2021	Justin Merryman, DC	Therapeutic procedure, 1 o...	97110	23.00	-		-
49457	03/01/2021	Justin Merryman, DC	Chiropractic manipulative ...	98942	51.75	-		-
53942	03/17/2021	Justin Merryman, DC	Therapeutic procedure, 1 o...	97110	23.00	-	)	
53942	03/17/2021	Justin Merryman, DC	Chiropractic manipulative ...	98942	51.75	-	)	-
59891	04/14/2021	Justin Merryman, DC	Therapeutic procedure, 1 o...	97110	23.00	-		-
59891	04/14/2021	Justin Merryman, DC	Chiropractic manipulative ...	98942	51.75	-		-
63396	04/28/2021	Justin Merryman, DC	Therapeutic procedure, 1 o...	97110	23.00	-		-
63396	04/28/2021	Justin Merryman, DC	Chiropractic manipulative ...	98942	51.75	-		-
TOTALS					**1,695.25**	**-**	**)**	
							BALANCE DUE	